NORTHGATE VALLEY MEDICAL CENTER

Department of Diagnostic Radiology
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RADIOLOGY REPORT

Report Status: FINAL

PATIENT NAME	MRN	DOB	AGE / SEX
WHITFIELD, DANIEL R.	006150482	03/11/1999	27 Y / M
ACCESSION #	**PATIENT CLASS**	**LOCATION**	**PRIORITY**
RAD26-1014-7720	Emergency - Trauma	ED - Trauma Bay 2	STAT - TRAUMA
REFERRING PHYSICIAN	**ORDERED**	**EXAM PERFORMED**	**REPORT SIGNED**
Thomas Kerrigan, MD (Trauma Surgery)	10/14/2026 23:06	10/14/2026 23:21	10/14/2026 23:58

EXAM: XR FEMUR RIGHT, 2 VIEWS (PORTABLE)

CPT 73552

CLINICAL HISTORY

27-year-old male, motorcycle collision at moderate speed. Obvious deformity of the right thigh with severe pain; unable to bear weight. Traction splint applied by EMS. Evaluate for femur fracture.

COMPARISON

None available.

TECHNIQUE

Portable AP and cross-table lateral radiographs of the right femur were obtained in the trauma bay. The hip and knee are included in the field of view. A traction splint partially overlies the proximal thigh.

FINDINGS

Bones: There is an acute, comminuted fracture of the mid-diaphysis (mid-shaft) of the right femur. The primary fracture line is oblique, extending from proximal-lateral to distal-medial over a length of approximately 6.5 cm. A separate wedge-shaped (butterfly) fragment measuring approximately 3.2 x 1.4 cm arises from the medial cortex and remains in contact with both main fragments. The distal fragment is displaced posteriorly and medially by approximately 1.8 cm, with 2.1 cm of overriding (shortening) and approximately 12 degrees of varus angulation.

Hip: The femoral head is located within the acetabulum. No displaced fracture of the femoral neck, intertrochanteric region or subtrochanteric region is seen, although evaluation of the femoral neck is limited by the portable technique and overlying traction splint.

Knee: No fracture of the distal femur, patella or proximal tibia. No fracture extension into the knee joint. No lipohemarthrosis on the cross-table lateral view.

Bone mineralization: Normal for age.

Soft tissues: Marked soft tissue swelling of the mid thigh. No subcutaneous gas or radiopaque foreign body to suggest an open fracture.

IMPRESSION

1. Acute, comminuted, displaced and overriding fracture of the right femoral mid-shaft with a medial butterfly fragment, 2.1 cm shortening and approximately 12 degrees of varus angulation (AO/OTA 32-B2, wedge pattern).
2. No hip dislocation or knee joint involvement. No displaced femoral neck fracture; however, ipsilateral femoral neck fractures can be occult in high-energy femoral shaft injuries. Dedicated AP pelvis and right hip radiographs, or CT, are recommended prior to fixation.
3. Marked soft tissue swelling without radiographic evidence of an open fracture.

CRITICAL RESULT: Findings were communicated to Dr. Thomas Kerrigan (Trauma Surgery) by telephone at 23:52 on 10/14/2026, with read-back confirmation.

Interpreted and electronically signed by:
Jonathan Mehta, MD
Attending Radiologist, Emergency & Trauma Imaging

Signed: 10/14/2026 23:58
Workstation: NVMC-RAD-07

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